

Primary Statement of Harm

*Criminal Referral — Misconduct in Public Office, Perjury, POCA, PCJ,
Safeguarding*

Case references

**BV20D01752 | CA-2024-001342 | AC-2025-LON-004293 | BL-2024-001217
CCR-6501-26-0100-000 | 0103/17APR26**

Referred to:

MPS Serious and Organised Crime Command
CPS Special Crime Division
NCA Economic Crime Command
Thames Valley Police Major Crime Unit

Nadia Zahmoul

Litigant in Person

18 March 2026 | Updated 25 May 2026

Veritas Numquam Perit

PRIMARY STATEMENT OF HARM

Item	Detail
Complainant	Nadia Zahmoul (nadia@rosekross.com)
Date of this Statement	18 March 2026 — updated 25 May 2026
Proceedings	BV20D01752 (Family); CA-2024-001342 (Court of Appeal); AC-2025-LON-004293 (JR); BL-2024-001217 (Chancery)
Addressed to	MPS Serious and Organised Crime Command; CPS Special Crime Division; NCA Economic Crime Command; Thames Valley Police Major Crime Unit
Nature of referral	Criminal referral — Misconduct in Public Office; Perjury Act 1911; POCA 2002; Perverting the Course of Justice; Safeguarding under Care Act 2014 s.42

This is not a family dispute. This is a criminal referral.

This referral concerns the unlawful revocation of court-ordered participation measures for a disabled adult litigant on 7 March 2024, and the systematic exploitation of that revocation to exclude her from legal proceedings affecting her financial survival, her housing, and the welfare of her children. The revocation has never been reversed. It constitutes wilful misconduct in public office under the common law (AG's Reference No 3 of 2003), a breach of the mandatory duty under EA 2010 s.149, and perverting the course of justice in that the revocation was used to prevent the Complainant from challenging false findings and fraudulent orders.

The harm is ongoing. The Complainant is currently homeless, has no income, is under crisis mental health care, and faces a default judgment of £1,008,848.42 accruing interest at £281.38 per day. This is a safeguarding matter under the Care Act 2014 s.42 as well as a criminal referral. Both duties are invoked simultaneously and independently. **As at the date of this updated statement, the safeguarding duty has been formally engaged: on 17 April 2026 the Metropolitan Police Service attended the Complainant in person and initiated a Care Act s.42 referral (ref 0103/17APR26), classifying her as an adult at risk on the police's own assessment of the documentary record.**

Every element in this referral has been raised with a competent authority. Every route has been closed without engagement with the evidence. This referral gives the police the authority that no other institution has yet exercised: to hold the responsible persons accountable.

1. THE PRIMARY ACT: UNLAWFUL REVOCATION OF PARTICIPATION MEASURES

The primary act from which every other harm flows is the unlawful revocation of court-ordered participation measures on 7 March 2024. Everything that follows in this referral — every false finding, every unilateral order, every costs order, every dismissed application, every hospitalisation — is a consequence of that single unlawful act.

Item	Detail
What happened	During the final hearing (19–28 February 2024), MacDonald J ordered participation measures for NZ under FPR Part 3A and PD3AA. These included: regulated questioning, frequent breaks, adjusted courtroom environment, assistance with the bundle. The measures were in force throughout the 10-day trial. On 7 March 2024 — six days after trial — NZ's legal team stepped down. The participation measures ceased. There was no order revoking them. No hearing. No reassessment under FPR Part 3A. No equality analysis. No recorded decision. No notification to NZ. No opportunity to respond.
What the law required	FPR Part 3A.9(1): <i>"The court's duties under rules 3A.3 to 3A.6 apply as soon as possible after the start of proceedings and continue until the resolution of the proceedings."</i> FPR Part 3A.9(2): <i>"The court must set out its reasons on the court order for — (a) making, varying or revoking directions referred to in this Part."</i> EA 2010 s.149 PSED: A mandatory, non-discretionary duty requiring active, visible consideration of disability in every decision affecting a disabled party. Silence does not discharge the duty. Autism Act 2009 statutory guidance: Public bodies must identify autistic adults, record their needs, and ensure adjustments are made. Where guidance is not followed, a good reason must be given.
What occurred instead	The measures were revoked without any of the required steps. No order was made. No reasons were recorded. NZ was never told the measures had ceased. She could not challenge what was never recorded. Every subsequent decision — the judgment (18 April 2024), the costs order (24 May 2024), the draft order process (April–May 2024), the 8 May 2025 judgment, the Court of Appeal refusal (23 December 2024), the JR refusal (11 March 2026) — was made while this unlawful revocation was in force and had never been corrected.
Duration	7 March 2024 to 25 May 2026 — 810 days and

	counting. The revocation has never been reversed.
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2. STATUTORY FRAMEWORK: WHY THIS IS A CRIMINAL MATTER, NOT A CIVIL DISPUTE

This section is addressed directly to the triage officer who receives this referral. The statutory framework establishes that this cannot be categorised as a family law dispute or a judicial conduct complaint. It is a criminal matter on three independent legal bases.

Legal basis	What it requires	Why it is criminal, not civil
FPR Part 3A and PD3AA	Court service and those administering proceedings must identify vulnerable parties, consider how their participation may be affected, and maintain appropriate measures until resolution of proceedings. Revocation requires a court order with recorded reasons. No order was ever made.	A public officer who removes a mandatory statutory protection from a disabled party, without order, without notice, without reasons, and without giving her the opportunity to respond — and who then makes findings of misconduct against her for the consequences of that removal — is not making a judicial error. That is wilful misconduct in a public office: AG's Reference (No 3 of 2003) .
EA 2010 ss.20-21, 29, 149	Service-providers including HMCTS and MoJ must make reasonable adjustments so disabled persons are not placed at a substantial disadvantage. The PSED is a continuing, non-discretionary duty requiring active, visible consideration of disability in substance, not form. NZ is a disabled person within s.6 EqA 2010 by reason of autism (CLAAS diagnosis June 2025, confirming lifelong condition), PTSD, and associated conditions.	EA 2010 s.149 is the proof of the wilful element, not the offence itself. A public officer who knows the statutory duty, has been placed on explicit notice of the disability (22 oral references at trial, CLAAS referral in progress by March 2025), and proceeds without complying — is not unaware. The absence of any disability reference across 111 paragraphs of judgment, and zero disability references in the costs judgment, is not an oversight. It is the visible record of a choice.
Autism Act 2009 statutory guidance	Public bodies must identify autistic adults, record their needs, and ensure adjustments are made to enable access to services. Treated as if issued under s.7 Local Authority Social Services Act 1970. Where guidance is not followed, a good reason must be given. No reason has ever been given for the	HMCTS has never provided a reason for the cessation of participation measures. When asked, it classified the matter as a "judicial decision" (14 November 2025) — no such decision exists. That misclassification is itself a false statement by a public officer to prevent investigation. Providing

	revocation or non-maintenance of NZ's participation measures.	a false administrative reason to close a complaint is a further act of MiPO.
Article 6 ECHR	The right to a fair hearing must be practical and effective, not theoretical or illusory. For disabled litigants this includes the right to effective participation, which may require positive steps to adjust procedures.	NZ has not had effective participation in any hearing since 28 February 2024. Every decision affecting her financial settlement, her housing, and her children has been made without her voice. The right guaranteed by Article 6 has been rendered illusory — not by accident, but by the unrevoked revocation of the only protections that made participation possible.
Common law — procedural fairness	Parties must have a fair opportunity to know and meet the case against them. Disability and vulnerability must be taken into account where relevant to participation.	NZ has never been given the opportunity to respond to the revocation of her participation measures. She was not told it had occurred. She could not challenge what was not recorded. This is the foundational procedural unfairness from which every downstream harm flows.
Care Act 2014 s.42	Where a local authority becomes aware that an adult with care and support needs may be experiencing abuse or neglect, it must make enquiries. Police are a statutory safeguarding partner. This duty is independent of the criminal investigation.	NZ is an autistic adult with PTSD, currently homeless, with no income, under crisis mental health care at Chelsea and Westminster Hospital. The withdrawal of court-ordered protective measures for a disabled adult, resulting in financial ruin and repeated hospitalisations, is abuse or neglect within s.42. This safeguarding duty was formally engaged on 17 April 2026 when MPS officers attended the Complainant in person and initiated a Care Act s.42 referral (ref 0103/17APR26).

3. ACTIVE AND ONGOING HARM: THE CURRENT POSITION

This is not a referral about historical events. The harm is occurring now. The following are the Complainant's current circumstances, stated in plain terms for safeguarding purposes.

- **HOUSING:** The Complainant is homeless. She stays in a friend's flat at address withheld. She has no tenancy and no permanent address.

- **INCOME:** The Complainant has no income. The MPS order was extinguished by para 34 of the final order. The pension transfers (£2.9 million) have not been made. The education fund (£700,000) is under H's sole control. She has received nothing under the financial settlement.
- **FINANCIAL LIABILITY:** A default judgment of £1,008,848.42 has been entered against her in BL-2024-001217 (Detach Lending Limited v Zahmoul). Interest accrues at £281.38 per day. She has no means to meet this.
- **HEALTH:** The Complainant has autism (CLAAS diagnosis June 2025), PTSD, Sjögren's syndrome (biopsy October 2025, active flare), and has experienced three hospitalisations in 2025 (March, June, November). She is under NHS clinical care. Her distress is directly and causally linked to the ongoing legal proceedings and the withdrawal of reasonable adjustments — this is recorded in writing by CLAAS (Dr Catherine Cheung, 1 July 2025).
- **PARTICIPATION:** The Complainant has not been able to participate effectively in any legal proceeding since 7 March 2024. Every decision made since that date — in the Family Division, the Court of Appeal, the Administrative Court, and the Chancery Division — has been made without her effective participation.
- **DAUGHTERS:** The Complainant's two daughters — Jade Mia (at university in Boston) and Lila Mae (at university in New York City) — are affected by the non-implementation of the financial settlement. H controls the education fund.
- **ESCALATION RISK:** Every day that passes without police engagement adds to an accruing judgment, deepens financial ruin, and prolongs the conditions that have produced three hospitalisations. Delay is itself harm.
- **SAFEGUARDING REFERRAL ACTIONED BY POLICE (17 APRIL 2026):** Officers of the Metropolitan Police Service attended the Complainant in person and initiated a safeguarding referral under Care Act 2014 section 42 (reference **0103/17APR26**). The referral was made on the police's own assessment of the documentary record. The Complainant is now formally classified as an adult at risk. This classification engages Duty 2 of this referral. Duties 1 (Crime Recording) and 3 (Intelligence Recording) remain to be discharged.
- **POLICE ARREST ON H'S REPORT (13 AUGUST 2025):** The Complainant was arrested by the MPS on a report made by H concerning alleged pawning of watches (CRN/CRIS 01/747777/24). The arresting officer, DC Anca Constantinescu (Public Protection — Community Safeguarding Unit Team 2, Charing Cross), professionally observed that NZ *"presented as distressed and vulnerable, with heightened anxiety, and required information to be repeated and clarified."* The investigation concluded without CPS referral. The pawnbroker waived all interest and charges. DC Constantinescu's letter of 5 October 2025 constitutes an independent MPS professional observation of NZ's vulnerability — corroborating the clinical record from a law enforcement source. It is available on request.

4. THE CAUSAL CHAIN: FROM REVOCATION TO RUIN

The following table sets out, step by step, how the unlawful revocation of participation measures on 7 March 2024 produced every downstream harm. Each link is documented. Nothing requires inference.

Step	What happened	Direct consequence	Evidence reference
1	7 March 2024: participation measures revoked without order, notice, or reasons.	NZ has no protections. No legal team. No participation framework. No income (MPS in default).	FPR Part 3A.9 — no revocation order exists. 71 documented MPS enforcement requests. Exhibit NZ1 pp.1-219.
2	April-May 2024: judgment and costs order produced. Disability entirely absent. Character findings substituted.	False record published: [2024] EWFC 79 and [2024] EWFC 114. NZ characterised as litigant of bad character. Foundation for all downstream decisions.	Zero disability references across 111 paragraphs. 22+ oral references at trial. Para 64 obsession finding. Costs judgment "stole", "disruptive", "inability to show restraint" — all documented autistic traits.
3	April-May 2024: six unilateral draft orders filed by Campbell KC without NZ's participation. Para 34 inserts MPS arrears remission on 3 May 2024.	£50,000 in MPS arrears extinguished. Live D50K enforcement application suppressed. NZ exits proceedings with £89.	D50K filed 2 May 2024. Karen Selby-King confirms delivery to MacDonald J. Campbell KC files para 34 one minute after NZ's Lord Chancellor email. Timing documented.
4	23 December 2024: Court of Appeal refuses permission to appeal on papers. Part 3A grounds dismissed as "not raised before judge" — factually false.	Appellate route closed. MacDonald J then proceeds on 8 May 2025 characterising all legitimate avenues as exhausted.	NZ1 pp.1-219 — 219 pages of documented requests from 7 March 2024. Moylan LJ's order does not engage with this evidence.
5	8 May 2025: MacDonald J proceeds in NZ's absence. Dr Poyser's letter (26 March 2025) confirming mental health crisis is misread as fitness to attend without support.	Set-aside dismissed. Contempt dismissed. MPS dismissed. Documents signed in NZ's absence. Indemnity costs. Decree absolute pronounced. Detach repayment triggered.	Dr Poyser letter 26 March 2025. CRHTT assessment 4 March 2025 (A&E admission, lorazepam, Haloperidol). CLAAS Report 1 July 2025 — distress attributed to withdrawal of adjustments.
6	31 March 2025: Default judgment entered in BL-2024-001217. NZ not served with claim form. Never knew proceedings had been issued.	£1,008,848.42 judgment. Interest at £281.38/day. No assets to meet it because financial settlement never implemented.	Detach proceedings causally linked to false family orders: no participation measures → excluded from draft order → Detach repayment triggered on

			conclusion of proceedings on H's unilateral terms.
7	11 March 2026: JR permission refused by Collins Rice J (TWM). HMCTS administrative failures conflated with judicial determinations — the claim is mischaracterised.	Last domestic court route closed. CPR 3.1(7) revocation filed 12 March 2026. Police referral is the only remaining route.	AC-2025-LON-004293 order 11 March 2026. CPR 3.1(7) supplementary note filed 12 March 2026.
8	17 April 2026: MPS officers attend the Complainant in person and initiate Care Act s.42 referral (ref 0103/17APR26). Adult-at-risk classification.	Safeguarding duty engaged on police own initiative. Documentary record assessed and accepted by law enforcement. Crime Recording and Intelligence Recording duties remain outstanding.	MPS safeguarding referral 0103/17APR26 — 17 April 2026. Made on police's own assessment of the documentary record.
NOW	25 May 2026: 810 days since revocation. Revocation never reversed. No court has determined whether it was lawful.	Homeless. No income. £1m+ liability. Under NHS clinical care. Adult at risk. Daughters affected. Judgment accruing at £281/day.	This referral is filed because no other route remains. The police have jurisdiction that no court has yet exercised.

5. THREE DUTIES THAT CANNOT BE SUPPRESSED — WHAT POLICE MUST DO

This section is addressed to the officer who receives this referral. Three independent legal duties arise on the face of the materials before you. Each is stated in binary form: either the action is taken, or a written explanation of why the threshold has not been met must be provided. Silence is not a lawful response.

Duty 1 — Crime Recording

Legal basis: Home Office Counting Rules (HOCR) and National Crime Recording Standard (NCRS). Where a victim makes a credible allegation of a criminal offence and there is no credible evidence to the contrary, the allegation must be recorded as a crime.

What is required: The Oxus perjury allegation meets the threshold on its face: a signed public SEC document (16 January 2024), a dated sworn testimony (22 February 2024), and three admissions under oath. This is a specific offence (Perjury Act 1911 s.1), a named

suspect, and documentary evidence. Once recorded, it must be assigned, reviewed, and actioned. Non-recording is a performance failure reportable to HMICFRS.

The binary: Either record this allegation as a crime under NCRS and assign it to an investigating officer. Or provide written confirmation that the NCRS threshold has not been met and the specific reason why.

Status as at 25 May 2026: Outstanding.

Duty 2 — Safeguarding under Care Act 2014 s.42

Legal basis: Care Act 2014 s.42: where an adult with care and support needs may be experiencing abuse or neglect, the local authority must make enquiries. Police are a statutory safeguarding partner. This duty is independent of the criminal investigation. It arises on the Complainant's circumstances alone.

What was required: NZ is an autistic adult with PTSD, currently homeless, with no income, under NHS clinical care. Court-ordered protective measures were removed without lawful authority. Three hospitalisations followed. CLAAS (Dr Catherine Cheung, 1 July 2025) records in writing that NZ's distress is attributable to the withdrawal of reasonable adjustments by the court.

The binary: Either make a safeguarding referral under Care Act 2014 s.42. Or provide written confirmation that the s.42 threshold has not been met and the specific reason why.

Status as at 25 May 2026: Duty 2 ENGAGED. On 17 April 2026, officers of the Metropolitan Police Service attended the Complainant in person and initiated a Care Act s.42 referral (reference **0103/17APR26**). The referral was made on the police's own assessment of the documentary record. The Complainant is now formally classified as an adult at risk.

Duty 3 — Intelligence Recording

Legal basis: MPS Intelligence Management System and NCA equivalent. All referrals must be recorded as intelligence regardless of whether a crime report is opened. Intelligence recording creates a searchable, permanent record.

What is required: H is connected to the Rakishev network, which is under active investigation by Thames Valley Police (Andrew Mountbatten-Windsor arrest, 19 February 2026), the SEC (TCR 17284-760-060-567), and IRS:CI (Special Agent Robert McGarry, NY Field Office, open investigation). Recording this referral as intelligence means it appears when those investigations generate queries about the same network.

The binary: Either record this referral as intelligence in the MPS/NCA intelligence management system and cross-reference with the Thames Valley Police Andrew investigation. Or provide written confirmation that intelligence recording has not occurred and why.

Status as at 25 May 2026: Outstanding.

Summary as at 25 May 2026: Duty 2 has been engaged by MPS's own action on 17 April 2026 (ref 0103/17APR26). Duties 1 and 3 remain outstanding.

6. WHAT THIS REFERRAL IS — AND WHAT IT IS NOT

This referral IS	This referral IS NOT
A criminal referral alleging specific offences with named suspects, specific dates, and documentary evidence.	A challenge to judicial decisions. Police cannot set aside a court order. That is not what is asked.
A safeguarding referral under the Care Act 2014 s.42 — engaged on 17 April 2026 by MPS on its own initiative (ref 0103/17APR26).	An appeal. Every appellate route has been exhausted.
An intelligence referral connecting H to a network currently under active investigation by Thames Valley Police, the SEC, and IRS:CI.	A request for police to adjudicate the financial settlement. The financial settlement is the consequence of the criminal conduct, not its subject.
A request for police to exercise independent jurisdiction — jurisdiction that no court has yet exercised — to investigate conduct that has been closed at every other route.	A complaint about litigation outcomes. The outcomes are the evidence of the criminal conduct, not the complaint itself.

The full evidential package supporting this Primary Statement of Harm is contained in the accompanying annexes. Each annex addresses one element of the referral, identifies the specific criminal offence, and states the binary action required.

The Complainant has autism and PTSD. She has Sjögren's syndrome (biopsy confirmed October 2025, active flare). She is filing this referral from a position of severe cognitive and physical depletion, after 810 days of unlawful exclusion from her own proceedings. The sophistication of this document should not create the false impression that she is managing better than she is. That warning is given explicitly by her treating clinician: Dr Catherine Cheung, CLAAS, 20 June 2025.

She is not asking police to rescue her. She is asking police to do their job.

Nadia Zahmoul • nadia@rosekross.com • Date: 18 March 2026 • Updated: 25 May 2026

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